RISPERIDONE

Raised prolactin

Behavioural symptoms in autism

CHEMICAL NAME 3-[2-[4-(6-fluoro-1,2-benzisoxazol-3-yl)piperidin-1-yl]ethyl]-2-methyl-6,7,8,9-tetrahydro-4H-pyrido[1,2-a]pyrimidin-4-one

CLASS Second-generation (atypical) antipsychotic (benzisoxazole)

TRADE NAMES Risperdal · Risperdal Consta (depot) · Risdone · Rispa

THERAPEUTIC USES / INDICATIONS

- Schizophrenia; bipolar mania.
- Persistent aggression in Alzheimer's dementia (short-term, max 6 weeks).
- Conduct disorder/irritability in autism (children ≥5 yr).
- Long-acting injection for maintenance.

ADMINISTRATION

- **PO** once or twice daily (tablets, liquid, orodispersible).
- **Depot (Consta):** deep IM every 2 weeks — oral cover for first 3 weeks.

DOSAGES

- **Schizophrenia:** 2 mg day 1, 4 mg day 2; usual 4–6 mg/day (max 16 mg).
- **Mania:** 2–6 mg daily.
- **Dementia aggression:** 0.25 mg twice daily, max 1 mg twice daily.
- **Autism (20–45 kg):** 0.25–0.5 mg daily.

PHARMACOKINETICS

- **Peak:** 1–2 h.
- Metabolised by CYP2D6 to active 9-hydroxyrisperidone (paliperidone).
- Excreted in urine. **t½:** 3–20 h (active fraction ~24 h).

PHARMACODYNAMICS

- **5-HT2A and D2** antagonist; dose-related EPS above ~6 mg; potent prolactin elevation.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Hyperprolactinaemia: galactorrhoea, amenorrhoea, gynaecomastia, sexual dysfunction.
- Weight gain, sedation.
- EPS at higher doses; dizziness, orthostatic hypotension.

ADVERSE EFFECTS (SERIOUS)

- NMS; tardive dyskinesia.
- Stroke and death in elderly with dementia.
- Metabolic effects; QT prolongation.
- Intraoperative floppy iris syndrome.

CONTRAINDICATIONS & CAUTIONS

- Hypersensitivity.
- **Caution:** dementia with Lewy bodies, Parkinson's, cardiovascular disease, renal/hepatic impairment.

KEY INTERACTIONS

- **CYP2D6 inhibitors (fluoxetine, paroxetine):** ↑ levels.
- **Carbamazepine:** ↓ levels.
- **Levodopa:** antagonism; **antihypertensives:** hypotension.

NURSING MANAGEMENT

- Monitor prolactin-related symptoms, weight, glucose, lipids.
- EPS assessment.
- Postural BP, falls.
- Dementia: review at 6 weeks.

PATIENT EDUCATION

- Report breast changes, missed periods, sexual problems.
- Rise slowly.
- Report stiffness or abnormal movements.

KEY POINTS

- Atypical antipsychotic; active metabolite paliperidone.
- Prolactin elevation.
- EPS dose-related (>6 mg).
- Dementia aggression short-term only.